جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : شادية عبدالرحيم حسن ابراهيم
المحافظة : القاهره **الحي / المدينة :** البساتين **العنوان :** 0
رقم البطاقة : 27008050104225 **رقم التليفون :** 01126558616 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 05-08-1970 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: شادية عبدالرحيم حسن ابراهيم
National ID: 27008050104225

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
شاديه
عبدالرحيم حسن ابراهيم
٢٢ ش فرن عبدالرسول-حسن التهامى
البساتين - القاهره
٢٥٠٤٢٢٥٠١٠٥٠٨٠٢٧٠
١٩٧٠/١٠/٢٥
JP8096119


٢٥٠٤٢٢٥٠١٠٥٠٨٠٢٧٠
٢٠٢٢/١٠
ربة منزل
أنثى
مسلمة
١
البطاقة سارية حتى ٢٠٢٩/١٠/١٥

Patient Name	شاديه عبد الرحيم حسن	Age	55
Sex		Special habits	
Family History		Comorbidity	
Surgical History		Telephone	01126558616
Menopausal Status	Post menopausal	Marital Status	married
ID	27008050104225	Medical ID	
Date	10-11-2025	Reason of MDT	Post operative

Medical Report	
Pathology: left BCS ER: 7/8 PR:6/8 HER2: 0 Ki67:10%	
NACT : S.M: Metsworkup: Free all margins ALN (1/13); extracapsular extension PT2 N1 Doctor's notes: Recommendations : For adjuvant therapy	
MDT DECISION	For: for ACX4T TAXOL WEEKLY X12 + PORT (11 CO ⑨ PLN ⑨ Arlle) Aramidex x 7 years + zometa /6m

Name	د/شيماء لاشين	د/ امنيه عيسوي	د/ نرمين رمضان
Signature		10/11/2025	Nermeen Ramadan 10/11/2025

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: شاديه عبد الرحيم حسن **MR. :** 4399	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 55-year-old individual with a history of left breast cancer diagnosed in October 2025. The tumor was treated with breast-conserving surgery and was pathologically classified as invasive ductal carcinoma, T2 N1, grade 1/3. Hormone receptor testing showed estrogen receptor 7/8 positive, progesterone receptor 6/8 positive, HER2 negative, and Ki-67 10 %. The initial adjuvant chemotherapy plan included four cycles of doxorubicin (Adriamycin) 105 mg/m² and cyclophosphamide (Endoxan) 1050 mg/m² (AC) followed by taxane therapy.

The patient's weight is 73 kg and height 150 cm. She has completed four cycles of AC and proceeded to taxane therapy for a planned 12-week course, receiving paclitaxel (Taxol) 135 mg in week 1 and week 2, with pharmacy dispensing for week 3. The patient has completed four cycles of AC.

During week 3 of the taxane regimen, the patient received paclitaxel 135 mg. Laboratory testing on 21 February showed an alanine aminotransferase (ALT) elevation to 2.5-fold the upper limit of normal; liver-support measures were initiated, and all other laboratory results were acceptable. The patient is currently in week 3 of Taxol therapy.

The patient sustained a tibia and fibula fracture requiring internal fixation. Consequently, chemotherapy was postponed until after the orthopedic surgical procedure.

The patient remains pending initiation of an aromatase inhibitor for a planned three-month duration, and a bone scan is still pending.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	:الأستاذه / شاديه عبد الرحيم حسن	Request No.	: 13001473
Patient No.	:27008050104225	Request Date	: 17/06/2025
Path No.	:2049/25	Verification Date	:24/06/2025
Sex	:Female	Financial Category	:جوستاف روسي - مبادره
Age	:54 Y	Department	:Out Patient
Referreing Doc :		Registered by	:.marwa mohamed youss

Pathology Report

Clinical Data
Left breast mass

Nature of specimen
Tru cut biopsy, collected at 17/6/2025, 10:30 am

Gross examination
Multiple grayish white tissue cores up to 2 cm, totally submitted.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
* DCIS: No
* Angioinvasion: Couldn't be detected in the received biopsy
*Perineural invasion: No
• IPT (Positive control showed appropriate reactivity) :
* ER: Strong nuclear reaction in 50 % tumor cells (Allred score 7/8)
* PR: Strong nuclear reaction in 30 % tumor cells (Allred score 6/8)
* HER2: No membrane reaction in tumor cells (score 0)
* Ki 67: 10 % nuclear reaction of tumor cells

Diagnosis
Left breast mass, Tru cut biopsy:

Verified by :
Dr.Heba Hasan

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / شاديه عبد الرحيم حسن	Request No.	: 13001473
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Age	: 54 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Invasive duct carcinoma, Grade 2
IPT:
* ER: Positive (score 7/8).
* PR: Positive (score 6/8).
* HER2: Negative (score 0).
* Ki67: Positive in 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim
Dr.Heba Hassan Adly

Verified by :
Dr.Heba Hasan

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY

Patient Name	: الأستاذه / شاديه عبد الرحيم حسن	Request No.	: 13003673
Patient No.	: 27008050104225	Request Date	: 04/10/2025
Path No.	: 3017/25	Verification Date	: 16/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 55 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

CLINICAL DATA:
Left breast cancer, UpFront.

NATURE OF SPECIMEN:
Left WLE & ALN specimens

GROSS EXAMINATION:
1- Left ALN: A fibro fatty tissue piece measured 8x7 cm & entangling 13 nodules up to 0.7x0.7cm.
2- Left WLE specimen measured 12x10x3.5cm , with covering skin elipse measured 7x2.5cm
• Tumor extent
o Number: A firm gryish white tumor bed site
o Size: 2.8x2.5x1.5 cm
o Skin involvement: Free grossly
• Distance from margins: 0.5cm from lateral margin (least margin)
Block designation :
1.Medial
2.lateral
3.superior
4.Inferior
5,deep
6,7,8 mass
,9,10,11,12. lymph nodes
Grossing By :Dr.Heba Hassan Adly

Microscopic examination:
Left breast ; WLE & ALN biopsy :
• Histological type :invasive duct carcinoma

Verified by :
Dr.Heba Hasan

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY

Patient Name	: الأستاذه / شاديه عبد الرحيم حسن	Request No.	: 13003673
Patient No.	: 27008050104225	Request Date	: 04/10/2025
Path No.	: 3017/25	Verification Date	: 16/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 55 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

•Histological grade:grade 2(differentiation 3 +pleomorphism 2+mitosis1)
• Ductal carcinoma in situ: High grade
• Skin involvement: Free
• Lymph node involvement: Positive (1/13) with extracapsular extension
• Angio invasion:Positive
• Perineural invasion: Positive
• Resection Margins: Free all resected margins
*Additional pathological findings : No

DIAGNOSIS:
Left breast ; WLE & ALN Biopsy :
- Invasive carcinoma grade II with high grade duct carcinoma insitu .
-Free all resected margin, free skin
-Positive ALNs (1/13) with extracapsular extension
-Stage : pT2, pN1 , pMx

Signature
Prof . Dr Gamal Abdel Atty
Dr.Menan Hosam

Verified by :
Dr.Heba Hasan

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Shadia Abd El Raheem Hassan Ibrahim	Date : 25/09/2025
Exam : MAMMOGRAPHY BOTH	Age : 55 Year(s)
Medical ID:	Pat. ID: 99880

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 55-year-old female with pathologically proven left breast cancer, re-assessment study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma with mottled appearance yet no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- LOQ, middle third , irregular speculated high density mass lesion with distortion yet no suspicious microcalcifications.
- **Ultrasound** demonstrated 4 o'clock, middle third, 7mm deep to the skin, irregular spiculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study measuring 27 x 19 mm.
- Normal skin/nipple/areola complex.
- Two pathological axillary LNs are noted showing globular shape and lost hilum showing cortical thickness of 7 mm. No infra or supra-clavicular pathological lymph nodes.

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 5): 6 o'clock suspicious mass lesion with ipsilateral two pathological level 1 axillary lymph nodes.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-Informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name: Shadia Abd El Raheem Hassan Ibrahim	Date : 30/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age.: 55 Year(s)
Medical ID: 1936/2025	Pat. ID: 99880

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST , ABDOMEN AND PELVIS

Reported case of breast cancer

- **Left breast spiculated mass measuring 3.1 x2.9 cm with architectural distortion and skin thickening**
- **Multiple bilateral enlarged axillary L. Ns ,the largest at left axilla 10x15mm .**
- **Mosaic perfusion of both lung fields.**
- Clear both lung fields.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average size cardiac size. No pericardial effusion detected.
- Normal appearance of the chest wall.
- **Left thyroid nodule .**

II Abdomen & pelvis:

- The liver is average size in size with homogenous CT texture. No focal lesions.
- Average size gall bladder with two gall stones , the largest 17 mm.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both suprarenals.
- Normal appearance of the other visualized pelvic organs.
- No abdominal or pelvic masses or collections.

II.Scanned skeleton:

- **L2 ,S2 and left iliac bone sclerotic lesions.**

Opinion:

- ***Left breast mass with axillary lymphadenopathy.***
- ***L2 ,S2 and left iliac bone sclerotic lesions for correlation with bone scan..***
- ***No pulmonary , or hepatic metastatic deposits***

DR/Reham Khedr,,,MD 30/7/2025

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : Shadia Abdulrahim Hassan Ibrahim
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 04-09-2025 11:20
Medical No. : 194495

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History:

- 55-year-old lady, with history of left breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen allover the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]